Name: Niki Birkner | DOB: 8/4/1999 | MRN: 84143669 | PCP: Melinda Ho Ye Kong, MD | Legal Name: Nicole Birkner

MR PELVIS WITHOUT CONTRAST (ABD PERFORMED)

Results

UCSF Medical Center
UCSF Benioff Children's Hospital

UCSF Nancy Friend Pritzker Psychiatry Building - MRI

MRN: 84143669
Patient Name: **Birkner, Niki**
Date of Birth: **8/4/1999**

Requesting Physician: **Amber Dhillon**
Accession Number: **10026138258**
Exam Date: **12/05/2025**

Exam(s): **MR PELVIS WITHOUT CONTRAST (ABD PERFORMED)**
Exam Status: **Final**

The imaging provider, Alexandra Sophia Gersing, MD, reviewed your images and created this report to communicate with your care team. Your care team may not have yet reviewed this report but will discuss these results with you at your next visit, through telephone, or MyChart messaging. If you do not have a visit scheduled and wish to schedule one, please reach out to your practice through MyChart or by calling the practice directly.

MR PELVIS WITHOUT CONTRAST (ABD RAD PERFORMED), MR HIP WITHOUT CONTRAST, RIGHT, MR SACRUM WITHOUT CONTRAST 12/5/2025 9:32 AM

CLINICAL HISTORY: concern for lower abdominal wall/muscle tear. History of sacral stress injury. Hip pain began 6-8 months.

COMPARISON: None.

TECHNIQUE: Multiple MRI sequences of the sacrum, right hip, and pelvis were acquired at 3 Tesla without intravenous contrast.

FINDINGS:

Labrum: Tear of the anterior superior labrum. No paralabral cyst formation.

Ligaments: Unremarkable.

Tendons: Iliopsoas, gluteal, hamstring, and adductor tendons demonstrate normal signal and morphology without evidence of strain or tear.

Osseous and cartilaginous structures: 6 mm T1 hypointense and T2 hyperintense lesion in the distal sacrum.

Mildly low T1 marrow signal most pronounced in the lumbar spine.

Normal bone marrow signal and normal morphology without evidence of dysplasia, edema, fracture, or avascular necrosis. Specifically, no acute sacral fracture.

Subtle cartilaginous irregularity of the superior acetabulum.

Sacroiliac joints and spine: The sacroiliac joints are unremarkable. Visualized portions of the lower lumbar spine are otherwise within normal limits.

Miscellaneous: No significant hip joint effusion. Narrowing of the ischiofemoral space bilaterally with feathery edema signal in the quadratus femoris muscle bellies. The visualized musculature otherwise demonstrates normal signal and morphology without evidence of strain or tear.

Small volume free fluid within the pelvis. 8 mm cystic lesion along the distal anterior urethra (series 7 image 33, series 6 image 18), nonspecific, and could reflect a small diverticulum or Skene gland cyst. Multiple bilateral ovarian follicles.

IMPRESSION:

Narrowing of the ischiofemoral space bilaterally with feathery edema signal in the quadratus femoris muscle bellies. Findings can be seen with ischiofemoral impingement.

Otherwise, no acute tendinous or muscle injury.

Small well-circumscribed T2 hyperintense lesion of the coccyx without aggressive features. Finding is nonspecific though may represent an atypical hemangioma, intraosseous cystic changes, or benign notochordal lesion. 6-12 month follow-up contrast-enhanced MRI can be obtained for further evaluation as clinically indicated.

Report dictated by: Rana Gordji, MD, signed by: Alexandra S Gersing, MD, PhD
Department of Radiology and Biomedical Imaging

Signed by: Alexandra Sophia Gersing, MD

Study Images

Show images for MR Pelvis without Contrast (ABD RAD Performed)

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